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Psychosocial experiences of adults living with attention-deficit/hyperactivity disorder: a qualitative systematic review protocol

A H M Fahad , N M Rabiul Awal Chowdhury, Salma Akter Urme

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Department of Anthropology, Comilla University, Cumilla, Bangladesh

Correspondence to
A H M Fahad;
ahmfahadcou@gmail.com

ABSTRACT

Background Attention-deficit/hyperactivity disorder (ADHD) is a widely recognised mental health condition that frequently persists into adulthood, significantly impacting emotional well-being, interpersonal relationships, education and employment. Adults with ADHD often report substantial psychosocial challenges, including stigma, emotional dysregulation and occupational impairment. However, current literature has primarily concentrated on clinical diagnosis, comorbidities and treatment outcomes with a disproportionate emphasis on quantitative symptom-based metrics. Consequently, there is a lack of synthesised evidence regarding the subjective psychosocial experiences of this population. This study aims to systematically synthesise qualitative evidence to develop a comprehensive understanding of how adults with ADHD navigate these psychosocial challenges. The findings will provide evidence-based information to inform the development of holistic clinical assessments and targeted psychosocial interventions that better address the lived realities of adults with ADHD.

Methods and analysis A systematic search will be conducted across three electronic databases: PubMed, Scopus and the Cochrane Library. Medical Subject Headings (MeSH) terms and keywords will be adapted according to the search syntax of each database. The Population–Exposure–Outcome framework will guide the definition of inclusion and exclusion criteria. Inclusion will be restricted to empirical, peer-reviewed qualitative studies published in English between 1 January 2010 and 31 December 2025 that explore the psychosocial experiences of adults (defined as ≥16 years) with a confirmed diagnosis of ADHD. Two reviewers will independently conduct study selection, data extraction and quality appraisal, with disagreements resolved through discussion or consultation with a third reviewer. Methodological quality will be assessed using the Joanna Briggs Institute Critical Appraisal Checklist for Qualitative Research. Data will be synthesised using Thomas and Harden's thematic synthesis approach.

Ethics and dissemination Ethical approval is not required as this review uses publicly available data from previously published studies. The findings will be disseminated through publication in a peer-reviewed journal and presentations at academic conferences.

PROSPERO registration number CRD420251159182

STRENGTHS AND LIMITATIONS OF THIS STUDY

⇒ This systematic review protocol is guided by the Preferred Reporting Items for Systematic Review and Meta-Analysis Protocols (PRISMA-P) to ensure methodological transparency and structured reporting.
⇒ A comprehensive search strategy will be employed across multiple electronic databases, including PubMed, Scopus and the Cochrane Library, to ensure a thorough identification of relevant qualitative literature.
⇒ The restriction to peer-reviewed studies published in English from 2010 onwards may introduce language and publication bias, potentially excluding relevant studies from other languages or earlier periods.
⇒ The synthesis of findings is inherently dependent on the methodological quality and reporting standards of the primary qualitative studies, which may vary across research contexts.

INTRODUCTION

Attention-deficit/hyperactivity disorder (ADHD) is a commonly recognised mental health disorder that is characterised by difficulties with attention, increased levels of restlessness and impulsivity, typically beginning in childhood.[1] However, for a substantial proportion of affected children, ADHD symptoms do not fully resolve with age and may persist into adulthood.[2] According to the World Federation of ADHD International Consensus Statement,[3] ADHD is no longer viewed solely as a childhood condition.[3 4] Evidence also suggests that up to 40% of individuals diagnosed with ADHD in childhood continue to experience symptoms into adulthood.[4] Although the prevalence of both persistent and symptomatic adult ADHD is observed to decline with advancing age,[5] it remains a condition of significant public health importance. However, it is important to note that persistent adult ADHD refers to cases with clear longitudinal diagnostic continuity and individuals who met full diagnostic criteria for ADHD

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in childhood, and in contrast, symptomatic adult ADHD represents a broader clinical construct, encompassing adults who experience clinically significant symptoms and functional impairment attributable to ADHD.[6] When adjusted for the global population structure in 2020, persistent adult ADHD was estimated to affect approximately 2.58% of adults worldwide, and symptomatic adult ADHD was present in around 6.76% of adults, representing a large number of affected individuals globally.[5] Research has consistently shown that ADHD is more prevalent among males compared with females.[2] Despite this substantial global burden and growing recognition of adult ADHD, a critical gap persists in the literature where a large body of quantitative research has effectively documented clinical symptoms and impairment; these studies often inadequately capture the subjective, nuanced ways in which adults themselves understand, experience and navigate the disorder in their everyday lives.

Consequently, adults who are diagnosed with ADHD face myriad psychosocial experiences in their daily life functioning. ADHD is increasingly understood as a complex condition arising from the combined influence of multiple genetic, environmental and psychosocial factors, rather than from a single cause.[3] Prior research suggests that ADHD is associated with challenges and distress across psychological and working life.[7–9] These challenges often extend beyond clinical symptoms to include difficulties in personal functioning and economic outcomes, particularly in relation to employment and job stability.[7 10] Longitudinal studies tracking individuals from childhood into adulthood, as well as cross-sectional investigations of adult populations, indicate that ADHD is associated with an elevated risk of unemployment.[11–13] Moreover, in workplace settings, individuals with ADHD are more likely to experience poorer job performance, frequent job changes, voluntary job leaving and dismissal compared with individuals without ADHD.[14] Despite growing evidence on adult ADHD burden, adults' psychosocial experiences of living with ADHD in everyday life remain insufficiently understood in a focused, experience-centred synthesis.

Prior qualitative evidence, such as interviews with 20 adults diagnosed with ADHD aged between 20 and 64 years, Oscarsson *et al* reported a range of social challenges linked to both ADHD symptoms and workplace demands.[8] Participants described common difficulties, such as inattention, forgetfulness and impulsivity, as well as less frequently recognised challenges, particularly problems related to emotional regulation.[8] In addition, adults diagnosed with ADHD often face psychological factors such as mood swings, depression, stress, sleep disturbance and anxiety.[2] However, these are often treated as comorbidities, which may complicate recognition and management.[2] Similarly, another study from Alghamdi *et al* among university students showed that, among university populations, ADHD sufferers have been linked with depressive and anxiety symptoms and poorer academic performance.[9] Further findings from a qualitative case study involving Indonesian elementary school teachers indicated that insufficient understanding of ADHD contributes to students' academic, social and behavioural problems, alongside stigma and negative labelling, underscoring the importance of teacher education and supportive school policies.[15] In a qualitative phenomenological study of parents of children with ADHD, Mesfin and Habtamu revealed pervasive psychosocial challenges, including stigma, emotional distress, financial pressure and marital strain, alongside the use of both inward and outward coping strategies to manage caregiving demands.[16] Furthermore, Chen *et al* stated that beyond symptoms, ADHD can affect functional outcomes such as employment participation; comorbid stress-related and mood conditions may increase vulnerability.[17] ADHD has also been linked to a higher risk of road traffic accidents, substance use disorders, suicidality and chronic physical illnesses. Over time, these factors may contribute to lower earning potential among individuals living with ADHD.[3 18–21]

Furthermore, particularly in adulthood, ADHD is linked to substantial psychosocial difficulties, many of which are compounded by late diagnosis. Katzman *et al*,[22] in a narrative clinical review of adult ADHD, reported that ADHD is highly comorbid with mood, anxiety, substance use and personality disorders, and that overlapping symptomatology substantially complicates diagnosis and management.[22] Through thematic analysis of interviews with adult-diagnosed women, Morgan demonstrated that late ADHD diagnosis is associated with cumulative psychosocial harm, including trauma, internalised ableism and unmet psychological care needs, despite diagnosis being perceived as validating.[23] Therefore, adults diagnosed with ADHD go through strong emotional reactions before accepting the diagnosis, suggesting that psychological support is essential during and after diagnosis.[24]

Additionally, several quantitative pieces of evidence also indicate adults' experiences, including a large questionnaire-based study (N=414), which illustrated that early recognition and stimulant treatment of ADHD, especially during childhood, was the strongest independent predictor of occupational participation in adulthood, regardless of psychiatric comorbidity or current treatment status.[13] A comparative survey study in Japan demonstrated that adults with ADHD show markedly greater psychosocial and occupational impairment, including comorbid mental health conditions, reduced work productivity and adverse effects on relationships and self-esteem, compared with non-ADHD adults.[25] Furthermore, a field study of 171 employees[26] found that adults with ADHD experienced higher levels of job burnout compared with non-ADHD employees, and that this relationship was significantly mediated by executive function deficits.[26] ADHD and psychological factors such as depression and stress have a significant relationship.[11 12 27] Studies[27] suggest that adults with ADHD are at an increased risk of depressive disorders. In another study,[28] involving 77 adults clinically diagnosed with

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ADHD, depressive outcomes were examined in relation to treatment history and cognitive-behavioural factors.[28]

However, the existing literature remains limited in several important ways. Much of the research on adult ADHD has focused primarily on clinical diagnosis, psychiatric comorbidity and treatment outcomes, with a strong emphasis on quantitative measurement and symptom-based assessment. Although individual qualitative studies have provided important insights, these findings remain scattered across diverse settings and topics and have not yet been systematically synthesised in a focused, experience-centred manner. This fragmentation results in a limited integrated understanding of adults' lived psychosocial experiences of ADHD, creating a critical gap in the evidence base required for comprehensive clinical management. While quantitative data are effective in measuring impairment, they often fail to capture the subjective, nuanced ways in which adults perceive, navigate, and cope with the challenges of ADHD in their personal, social and occupational lives.

Addressing this evidence gap is therefore imperative for multiple audiences, most notably for clinical practice. For clinicians and mental health professionals, a robust synthesis of adults' lived psychosocial experiences can facilitate a move beyond traditional, symptom-based diagnostic frameworks towards more holistic and accurate person-centred assessments. Specifically, such an evidence synthesis can inform the development and refinement of targeted psychosocial interventions that address the daily functional and emotional challenges, including workplace difficulties, relationship strain and internalised stigma, which are frequently overlooked in standard clinical care.

For researchers, this review will help to identify critical dimensions of the adult ADHD experience that are insufficiently captured by existing comorbidity profiles alone, thereby highlighting priorities for future investigation. Therefore, the purpose of this qualitative evidence synthesis is to synthesise research on the psychosocial and everyday life experiences of adults with ADHD, with the explicit goal of generating actionable insights to inform future research, clinical practice and person-centred approaches to support and care.

Study aim

1. To systematically identify and synthesise qualitative evidence on the psychosocial experiences of adults living with ADHD.
2. To develop a comprehensive understanding of how adults with ADHD experience and navigate psychosocial challenges in everyday life, including social, emotional and occupational contexts.

METHODS

Protocol and registration

The protocol is registered in the PROSPERO International Prospective Register of Systematic Reviews (CRD420251159182).[29] The Preferred Reporting Items for Systematic Review and Meta-Analysis Protocols (PRISMA-P) guideline has been maintained to structure the protocol (online supplemental file PRISMA-P Checklist).[30] The systematic review will be conducted within a predefined timeframe, commencing in June 2026 and anticipated to conclude in December 2026, in accordance with established systematic review procedures.

Design

A qualitative narrative synthesis design will be used for this systematic review.

Eligibility criteria

Inclusion and exclusion criteria

Types of participants

This review will include studies with participants who have a confirmed diagnosis of ADHD. Participants will be selected based on the recognised diagnostic criteria, including Diagnostic and Statistical Manual of Mental Disorders, Fifth Edition (DSM-5)/ Diagnostic and Statistical Manual of Mental Disorders, Fifth Edition, Text Revision (DSM-5-TR), Diagnostic and Statistical Manual of Mental Disorders, Fourth Edition (DSM-IV), Diagnostic and Statistical Manual of Mental Disorders, Fourth Edition, Text Revision (DSM-IV-TR), International Classification of Diseases, Tenth Revision (ICD-10)/ International Classification of Diseases, Eleventh Revision (ICD-11) definitions, or clinician-confirmed medical record diagnosis. Studies wherein the diagnosis was confirmed through a clinician's assessment of medical records or via structured/semistructured diagnostic interviews such as Diagnostic Interview for ADHD in Adults, Fifth Edition (DIVA-5), Mini International Neuropsychiatric Interview (MINI), Structured Clinical Interview for DSM Disorders (SCID), Schedule for Affective Disorders and Schizophrenia for School-Age Children (K-SADS), will also be eligible for inclusion (K-SADS) will also be considered eligible. The eligible participant age range will be defined as 16 years and older to capture the psychosocial experiences of both young and established adults. While studies focused solely on paediatric populations will be excluded, those reporting on mixed-age samples will be included if the data for participants aged 16 and above can be distinctly analysed or if this age group constitutes the majority of the sample. Children, infants and older adults (aged over 65) will be excluded to ensure a focused synthesis on the core adult life-course and to maintain methodological homogeneity.

Types of studies

Eligible studies will be restricted to peer-reviewed, empirical qualitative research published in the English

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language. Qualitative studies include, but are not limited to, study designs such as phenomenology, grounded theory, ethnography and action research. Mixed-method studies will be deemed eligible if their qualitative components and associated findings can be clearly and independently extracted for synthesis. Literature based on quantitative data, particularly observational studies such as cross-sectional, case–control, cohort, systematic reviews, meta-analyses, scoping reviews, commentaries, conference abstracts, editorials and opinion pieces will be excluded from the review. We will also exclude all clinical trials, randomised controlled trials (RCTs), cluster-RCTs and quasi-RCTs, as this study area includes public health research.

The search will be restricted to studies published between 2010 and 2025 to capture contemporary literature reflecting the evolving recognition of ADHD as a lifespan condition, particularly following the transition towards DSM-5 diagnostic conceptualisations. In addition, recent decades have seen increasing qualitative attention to the psychosocial and lived experiences of adults with ADHD within changing social and technological contexts. Preliminary scoping searches also indicated that the majority of relevant qualitative studies were published after 2010, supporting the appropriateness of this time frame.

Types of phenomena of interest

The central phenomenon of interest for this review encompasses the psychosocial experiences of adults diagnosed with ADHD.

OUTCOME MEASURES

Primary outcome

Primary outcomes will be to identify the key themes in the psychosocial and lived experiences of adults living with ADHD, including experiences in social life, education and work and how they cope with daily challenges and social difficulties. For this protocol, 'psychosocial experiences' is defined as a broad, multidimensional construct that includes the interplay between an individual's psychological processes, such as self-perception, emotional regulation, coping mechanisms, etc, and their social-contextual factors, including interpersonal relationships, occupational functioning, educational attainment, etc. Rather than predefining specific factors, this review will employ an inductive approach, consistent with qualitative narrative synthesis, where the key themes and constituent factors of the psychosocial experience will be systematically derived directly from the narratives and interpretations reported in the included primary studies.

Secondary outcome

The secondary outcome of this review is to deliberate on the implications of the review findings for future research, clinical practice and policy related to adult ADHD.

SEARCH STRATEGY

A preliminary search of the PROSPERO international prospective register of systematic reviews was conducted to ensure the novelty of the review topic and to avoid duplication of existing research. As the review team has already ensured that, we then developed our search strategy and the selection of electronic databases. Every team member was associated with the strategic plan.

A comprehensive search of several international bibliographic databases will be conducted to identify all relevant published literature. The criteria for including and excluding articles will be set before identifying those relevant articles that match the review's objective. A useful way to clarify objectives, pinpoint key terms and create set inclusion and exclusion criteria is the Population, Exposure and Outcome (PEO) framework.[31] The PEO framework[31] will be used to identify search terms using Boolean operators (AND/OR/NOT). A detailed PEO-based search term is in table 1.

A comprehensive search of three core electronic bibliographic databases, PubMed, Scopus and Cochrane Library, will be conducted. These databases have been selected for their extensive and authoritative coverage of international biomedical health and psychological literature, ensuring a robust foundation for identifying relevant studies. The search will be restricted to studies published on or before 31 December 2025. In addition, different search strategies will be used for each database, where we will use a combination of Medical Subject Headings (MeSH) terms and keywords, depending on the search methods of the databases. To further enhance the comprehensiveness of our search and mitigate potential publication bias, the primary database search will be supplemented by a thorough review of the reference lists of all included studies (snowballing) and by conducting forward citation searches on key included articles. An example of a search strategy for the PubMed database is added in table 2.

STUDY SELECTION

Following the comprehensive database search, all retrieved records will be collated and imported into Zotero bibliographic software (V.8.0) to facilitate the identification and removal of duplicate entries. Following the removal of duplicates, the remaining unique records will undergo a three-stage screening process, which will be conducted independently by two reviewers (AHMF and SAU). In the initial stage, titles will be screened against the predefined eligibility criteria. In the second stage, the abstracts of all retained records will be reviewed to determine their relevance and suitability for full-text assessment. The final stage will consist of a detailed examination of the full texts of all potentially eligible studies to confirm their final inclusion. A log of all studies excluded during the full-text review, along with the specific reasons for their exclusion, will be

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Table 1 Detailed PEO-based search term

PEO acronym	Focuses area	Subject terms used in database search
Population	Adults and young adults (≥16 years) are diagnosed with ADHD	"Attention Deficit Disorder with Hyperactivity"(Mesh)) OR "ADHD"(Title/Abstract)OR "Attention Deficit Hyperactivity Disorder"(Title/Abstract)*AND ("Adult"(Mesh))OR "Young Adult"(Mesh)) OR adult(Title/Abstract)OR "young adult"(Title/Abstract)OR adolescent*(Title/Abstract))** NOT ("Child"(Mesh))OR "Infant"(Mesh))OR "Aged"(Mesh)*)
Intervention/Exposure	Psychosocial factors related to ADHD	*"Psychosocial Factors"(Mesh))OR "Social Environment"(Mesh))OR "Social Factors"(Title/Abstract)OR "Psychological Factors"(Title/Abstract)OR psychosocial (Title/Abstract)OR "lived experience"(Title/Abstract)**
Outcome	Lived experiences and perceptions of ADHD	*"Qualitative Research"(Mesh))OR qualitative(Title/Abstract)OR phenomenology(Title/Abstract)OR "grounded theory"(Title/Abstract)OR ethnography*(Title/Abstract)OR "thematic analysis"(Title/Abstract)OR experience*(Title/Abstract)OR perception*(Title/Abstract)**

ADHD, attention-deficit/hyperactivity disorder; PEO, Population, Exposure and Outcome.

meticulously maintained. Any disagreement between the two reviewers will be resolved through discussion, and if consensus cannot be reached, a third reviewer (NMRAC) will be consulted to make the final decision. In addition, supplementary search strategies, including snowballing and forward citation searching, manual screening of reference lists of included studies, and targeted searches to identify additional literature, will be conducted to identify any additional relevant articles that may not have been captured through the initial database searches. Any duplicates identified through this process will be removed, and the overall

Table 2 Search strategy example (PubMed)

Search step	Search query
#1	"Attention Deficit Disorder with Hyperactivity/psychology"(Mesh)
#2	("Attention Deficit Disorder with Hyperactivity"(Mesh)) OR (ADHD(tiab))OR "attention deficit hyperactivity disorder"(tiab))OR "attention-deficit/hyperactivity disorder"(tiab))OR "attention deficit disorder"(tiab))OR "adult ADHD"(tiab))
#3	(((((((("psychology" (Subheading))OR "Psychology"(Mesh)) OR "Social Factors"(Mesh)) OR "Socioeconomic Factors"(Mesh)) OR "Social Stigma"(Mesh)) OR "Quality of Life"(Mesh)) OR "Stress, Psychological"(Mesh)) OR "Life Change Events"(Mesh)) OR "Personal Narratives as Topic"(Mesh)) OR (psychosocial(tiab)) OR "psychosocial factor*"(tiab))OR "social factor*"(tiab))OR "family factor*"(tiab))OR "socioeconomic factor*"(tiab))OR stigma(tiab))OR stress*(tiab))OR coping(tiab))OR "daily functioning"(tiab))OR "social functioning"(tiab))OR relationship*(tiab))OR workplace(tiab))OR employment(tiab))OR education(tiab))OR experience*(tiab))OR "lived experience*"(tiab))OR perception*(tiab))OR perspective*(tiab))OR view*(tiab))OR meaning(tiab))OR narrative*(tiab))
#4	(("Adult"(Mesh)) OR "Young Adult"(Mesh)) OR (adult*(tiab))OR young adult*(tiab))OR adulthood(tiab))
#5	(((((("Interviews as Topic"(Mesh))) OR "Qualitative Research"(Mesh)) OR "Focus Groups"(Mesh)) OR "Narration"(Mesh)) OR (Qualitative(tiab))OR interview*(tiab))OR "focus group*"(tiab))OR phenomenology*(tiab)) OR ethnography*(tiab))OR "thematic analysis"(tiab))OR "content analysis"(tiab))OR "framework analysis"(tiab)) OR "grounded theory"(tiab))
#6	(((#2) AND (#3)) AND (#4)) AND (#5)

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study selection process will be documented and transparently reported in a PRISMA flow diagram (online supplemental file, table S1).

RISK OF BIAS (QUALITY) ASSESSMENT

The methodological quality of the included studies will be assessed using the Joanna Briggs Institute (JBI) Critical Appraisal Checklist for Qualitative Research.[32 33] This tool is specifically designed to evaluate the methodological rigour, credibility, and relevance of qualitative studies based on empirical data. The checklist consists of 10 items that assess key methodological domains, including the congruity between the research methodology and the research question, the adequacy of data collection methods, the appropriateness of data analysis, the representation of participants' voices, and the coherence between data, analysis and interpretation.

Each included study will be independently appraised by two reviewers (AHMF and SAU). For each item, studies will be rated as 'yes', 'no', 'unclear' or 'not applicable', in accordance with JBI guidance. Any discrepancies between reviewers will be resolved through discussion, and if consensus cannot be reached, a third reviewer (NMRAC) will be consulted to make the final decision. This process will help to ensure transparency and consistency in the quality appraisal. The results of the quality assessment will be used to inform the interpretation of the review findings, but will not be used as a basis for excluding studies. Additional information will not be sought from study authors if methodological details are unclear or missing in the published reports, and assessments will be based solely on the information available in the studies included.

DATA EXTRACTION

For the included studies, a structured data extraction form will be developed. Data will be extracted using the JBI Qualitative Assessment and Review Instrument, which is a standardised tool recommended for the organisation and synthesis of qualitative evidence.[34] Two reviewers (AHMF and SAU) will independently extract data from each included study, and the extracted information will be cross-checked for accuracy. Any conflict will be resolved through discussion, with the involvement of a third reviewer (NMRAC) where necessary. The data extraction form will capture key study characteristics, including the research methodology, phenomena of interest, study setting, geographical and cultural context, participant characteristics, data analysis methods and main findings. Additional bibliographic and descriptive information will also be recorded, such as author details, publication year, journal name, study objectives, study design, sample size and other relevant contextual information necessary for the synthesis.

DATA SYNTHESIS

The synthesis of qualitative data will be conducted following the three-stage thematic synthesis approach as described by Thomas and Harden.[35] The synthesis process will involve several structured and iterative steps to ensure rigour and depth of analysis. In the initial stage, all relevant findings from the included studies will be examined through line-by-line coding. This process will involve careful reading of the findings sections of each study to identify meaningful units of text related to the review objectives. Initial codes will be generated inductively to reflect participants' experiences, perceptions and interpretations. Coding will be iterative, and data segments may be assigned more than one code where appropriate to capture the complexity of the data. In the second stage, related codes will be compared and grouped to form descriptive themes. These themes will summarise patterns across studies and provide an organised description of the key psychosocial experiences reported by adults living with ADHD. During this stage, the themes will be refined through repeated comparison with the original data to ensure that they accurately represent the underlying findings.

In the final stage, analytical themes will be developed by interpreting the descriptive themes in relation to the review questions. This stage will move beyond description to generate higher-level explanations and insights into how adults experience, understand and cope with ADHD in their daily lives. NVivo V.14 software will be used to support data management, coding and theme development throughout the analysis process, helping to organise the data systematically and enhance transparency.

Patient and public involvement

None.

DISCUSSION

Adults living with ADHD frequently encounter significant challenges that adversely affect their emotional well-being, interpersonal relationships, education and employment. The literature indicates that experiences of stigma, difficulties with emotional regulation and impairments in occupational functioning are commonly reported. ADHD exerts a marked influence on occupational functioning in adulthood.[36] The core symptoms of the disorder, such as impaired concentration and difficulties with executive functions like organisation and time management, can directly undermine work performance and productivity. Consequently, adults with ADHD often report feeling persistently overwhelmed by routine responsibilities, which may lead to strained professional relationships and frequent job changes. As a result, frequent job changes are common among adults who are diagnosed with ADHD.[13] Transitions to new roles typically involve heightened expectations for self-discipline, organisational skills and emotional regulation.[25] Over time, these sustained demands can increase vulnerability to

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occupational burnout. This risk is compounded not only by the intrinsic features of ADHD but also by the high prevalence of coexisting mental health conditions.[13 25 26 37] The findings may inform more person-centred clinical practice and service planning. A better understanding of lived experiences may support the development of interventions that address psychosocial needs alongside clinical management. The review may also identify gaps in current research and highlight priorities for future qualitative inquiry.

To our knowledge, this will be the first systematic review to synthesise qualitative evidence on the psychosocial experiences of adults diagnosed with ADHD. Existing research related to ADHD has mainly focused on its symptoms, prevalence, comorbidities and treatment outcomes. There remains a limited synthesis of qualitative findings that reflect how adults experience ADHD in everyday life, and this review aims to address this gap.

This review has potential limitations. Only peer-reviewed studies published in English from 2010 onwards will be included, which may introduce language and publication bias. Although a comprehensive search strategy will be applied, some relevant studies may not be identified. The conclusions will depend on the methodological quality of the included studies. Despite these limitations, this review will provide a transparent synthesis of qualitative evidence and contribute to a more comprehensive understanding of adult ADHD.

Study status

This review protocol was prospectively registered with PROSPERO before the commencement of the systematic review process. At the time of protocol submission, the review was in its preliminary stages, with only the development of the search strategy and initial screening procedures underway. No data extraction, quality appraisal or data synthesis had been conducted. The protocol remains open to refinement, and any amendments arising from peer review will be transparently documented and justified in the final review. The study is projected to be completed by the end of 2026.

Amendments

Any modifications to this protocol will be explicitly documented, with a clear description of the changes and the rationale for each amendment. The revised version will also be updated and recorded in the PROSPERO register to ensure transparency, including details of the previous version of the systematic review protocol.

Ethics and dissemination

Ethical approval is not required for this review because it is based on a retrospective analysis of publicly available data and involves the secondary use of previously published studies. The findings will be disseminated through publication in a peer-reviewed journal and presented at relevant conferences and academic meetings.

Contributors AHMF is the guarantor and is responsible for the overall content of this work. AHMF was responsible for drafting the protocol and registering it in PROSPERO. SAU and NMRAC made essential contributions to the protocol. They significantly contributed towards the development of the search strategy, inclusion/exclusion criteria, selection of the appropriate study design, methods and tools. All authors read, revised and approved the final manuscript. Artificial intelligence tools were used only for language and grammar checking during the preparation of this manuscript. Specifically, ChatGPT (OpenAI) and Grammarly were used to assist with improving sentence clarity and correcting grammatical errors. No AI tools were used for generating research ideas, interpreting data, writing the scientific content or producing any original intellectual contribution. All conceptualisation, analysis and writing of the manuscript were carried out entirely by the authors, who take full responsibility for the accuracy and integrity of the work.

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ORCID iD

A H M Fahad https://orcid.org/0009-0005-8833-4615

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